# **Country Profile:** Vietnam



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# Country Overview

## Population and Demographics

| | | | |
|---|---|---|---|
| **Capital** | Hanoi | **Official Language** | Vietnamese |
| **Area** | 331,212 km² (127,882 sq mi) | **Currency** | Vietnamese Dong (VND) |
| **GDP (PPP) Total** | Approximately $1.46 trillion (2024, CIA World Factbook methodology) | **GDP (PPP) Per Capita** | Approximately $14,400 (2024) |
| **Government** | Single-party socialist republic under the Communist Party of Vietnam | **National Assembly** | 500 members (16th National Assembly, convened April 2026) |

Approximately 101 to 102 million people live in Vietnam. Current estimates from the World Bank, the UN Population Division, and Vietnam's General Statistics Office cluster in the 100.3 to 102.3 million range. Vietnam is the 15th most populous country in the world and the third most populous in Southeast Asia after Indonesia and the Philippines. Population growth has slowed to under 1 percent annually as fertility has fallen close to replacement level, and the country is urbanizing quickly, with roughly 40 percent of residents now living in cities, concentrated around Hanoi in the north and Ho Chi Minh City in the south. The median age is around 33 years, and Vietnam is often described as being in a closing demographic window: it still has a large working-age population, but the share of citizens over 65 is rising fast enough that planners expect the country to become an aging society within roughly two decades, well before it reaches high-income status. That timing matters directly for healthcare planning, since it compresses the period in which Vietnam can build chronic-disease and elder-care capacity before demand accelerates.[1,2,3,4,5]

## Government

Vietnam is a one-party socialist republic governed by the Communist Party of Vietnam (CPV), which controls the state apparatus through a leadership structure often called the 'four pillars': the CPV General Secretary (the country's most powerful figure, currently To Lam), the President (head of state), the Prime Minister (head of government), and the Chairman of the National Assembly. The National Assembly is constitutionally the highest organ of state power and the sole body that makes law, ratifies treaties, and approves the state budget, though in practice its agenda and personnel are shaped heavily by the Party. The 16th National Assembly, elected in 2026 and convened that April, has 500 members serving five-year terms, an increase from the 498 seats of the outgoing 15th National Assembly reflecting adjustments to electoral districts. Administratively, Vietnam has been mid-reform: in 2025 the government carried out a sweeping consolidation of provincial and district-level administrative units, merging dozens of provinces and cities to cut administrative layers and reduce overlapping bureaucracy, a change that also reshaped how provincial health

departments and hospital networks are organized. The CPV's dominance gives Vietnam unusual policy continuity and the ability to execute long, multi-decade plans, such as its current healthcare reform roadmap, without the disruption of contested elections, but it also means that reform speed depends heavily on internal Party consensus rather than public debate.[5,6,7]

## Economy

Vietnam's economy is one of the fastest growing in Asia, built on export oriented manufacturing, large inflows of foreign direct investment, and a young, increasingly skilled labor force that has made the country a preferred alternative to China for electronics, textile, and footwear production. On a purchasing power parity basis, the CIA World Factbook puts Vietnam's GDP at approximately 1.46 trillion dollars using 2024 data, which reflects the Factbook's standard PPP conversion approach. A separate, frequently cited projection places 2026 GDP (PPP) at roughly 2.03 trillion dollars, but that figure comes from IMF style databases that use updated PPP conversion factors and rebased national accounts rather than the Factbook's methodology; the apparent 39 percent jump between the two numbers is almost entirely a methodological artifact from switching PPP bases and comparison years, not a real one-year surge in output. Real GDP growth has generally run in the 6 to 7 percent range in recent years, driven by manufacturing and processing exports (notably electronics, machinery, and garments), a construction and real estate sector recovering from a 2022 to 2023 credit crunch, and steady FDI commitments from investors such as Samsung, Intel, and a growing base of manufacturers relocating supply chains out of China. Inflation has stayed moderate by regional standards, and the government has continued to pursue trade diversification through free trade agreements, including the CPTPP and the EU-Vietnam FTA, to reduce dependence on any single export market. Rising middle-class incomes are also expanding domestic demand for private healthcare, insurance products, and higher-quality medical services, which is central to why the private hospital sector has drawn so much investor interest.[4,8,9]

## Disease Prevalence

Vietnam is in the middle of an epidemiological transition: noncommunicable diseases (NCDs) now account for the large majority of deaths, while the country still manages a meaningful residual burden of infectious disease. Cardiovascular disease is the leading cause of death, followed by cancer (with liver, lung, and stomach cancers especially prevalent, partly reflecting high rates of hepatitis B infection, smoking, and alcohol consumption) and diabetes, which has risen sharply alongside urbanization, dietary change, and rising obesity rates. Chronic respiratory disease and hypertension are also widespread and under-diagnosed, particularly in rural areas with limited access to screening. On the infectious disease side, Vietnam continues to manage dengue fever outbreaks that intensify during the rainy season, a tuberculosis burden that remains among the higher in the Western Pacific region despite a strong national TB program, and a hepatitis B prevalence high enough that it is a leading driver of the country's liver cancer rate. Road traffic injuries also represent a significant and often underappreciated share of the disease and disability burden given Vietnam's motorbike dominated transport system. This mixed burden, chronic disease management on one hand and residual infectious disease control on the other, is a major reason the government's free checkup and screening push is being framed as a prevention strategy rather than only an access to treatment one.[10,11,12]

# Vietnam Health System

## Public Healthcare System

Vietnam's public health system is organized in a three-tier pyramid: central-level hospitals in Hanoi and Ho Chi Minh City that handle the most complex and specialized cases, provincial general and specialty hospitals, and a dense network of district hospitals and commune health stations that provide primary care and basic treatment close to where people live. This structure has given Vietnam relatively strong reach into rural areas compared to many peers at a similar income level, but it also produces a well documented capacity mismatch: central hospitals in the two largest cities are chronically overcrowded, sometimes housing two or three patients per bed on busy wards, while district and commune facilities often go underused because patients bypass them in favor of higher-tier hospitals they perceive as more reliable. Social health insurance, run by Vietnam Social Security, now covers roughly 93 percent of the population, a substantial expansion over the past decade driven by mandatory enrollment for employees, subsidized premiums for the poor and near-poor, and near universal coverage for children and the elderly. Coverage breadth has not eliminated financial risk, however: co-payments, drugs and services outside the insurance formulary, and the practice of seeking care at higher, costlier tiers still expose many households to significant out-of-pocket spending, which is precisely the gap the government's newer free-care initiatives are designed to close.[13,14,15]

## Private Healthcare System

Vietnam's private hospital sector is small relative to the public system but growing quickly and attracting substantial domestic and foreign capital, as rising incomes create demand for shorter wait times, more comfortable facilities, and internationally benchmarked quality of care. Leading domestic players include Vinmec (part of the Vingroup conglomerate), Hoan My Medical Corporation, and Tam Anh Hospital, while FV Hospital in Ho Chi Minh City has long been one of the most prominent foreign invested facilities serving both local patients and the expatriate and medical tourism market. Consolidation and cross-border investment have accelerated: Singapore based Thomson Medical Group agreed in 2023 to acquire FV Hospital's parent company for approximately 381.4 million dollars, one of the largest healthcare M&A deals in Vietnam's history, and that transaction closed in January 2024 after regulatory approvals were finalized. Deals of this size signal growing investor confidence that Vietnam's private healthcare market has reached a scale and regulatory maturity that justify major capital commitments, and they are part of a broader pattern of regional hospital groups, from Singapore, Thailand, and Japan in particular, building or acquiring positions in Vietnam rather than simply treating it as a source of outbound medical tourists. That said, private hospitals still serve a small minority of total patient volume, concentrated in the two largest cities and skewed toward wealthier and insured patients, so their influence on the overall health system is currently more about setting quality benchmarks and absorbing complex or elective cases than about bearing a large share of national care delivery.[16,17,18]

## Healthcare Financing

Vietnam's current health expenditure ran to roughly 4.6 percent of GDP in 2022, consistent with World Bank and WHO Global Health Expenditure Database figures for that year and broadly in line with regional peers at a similar income level, though still below the share typically seen in high-income health systems. Financing is split across three main sources: government spending, funded by general revenue and channeled substantially through the social health insurance system, covers close to half of total health spending; out-of-pocket payments by households account for roughly 40 percent, a level the government considers too high because it exposes families to the risk of catastrophic health spending; and external or donor financing makes up a small residual share that has been declining as Vietnam's income has risen and traditional development partners have scaled back direct health aid. The heavy reliance on out-of-pocket spending, even with insurance coverage extended to over 90 percent of the population, is the central financing problem the government's free-checkup and free-hospital-care initiatives are meant to address, since coverage on paper has not translated into full protection from costs at the point of care. Closing that gap is expected to require a substantial increase in mandatory insurance contributions, tighter government budget allocations to health, or both, which is why the planned rewrite of the Law on Health Insurance between 2030 and 2035 is treated as a financing reform as much as a coverage reform.[19,20,21]

## Workforce

Vietnam has made steady progress expanding its health workforce, but density and distribution remain constraints on system performance. Physician density is estimated at roughly 9 to 10 doctors per 10,000 population, below the levels seen in wealthier Asian health systems such as Japan, South Korea, or Singapore, and nursing density lags physician growth, creating a workforce mix that is comparatively doctor-heavy and nurse-light relative to international norms. The more persistent problem is geographic distribution: central hospitals in Hanoi and Ho Chi Minh City are well staffed with specialists, while rural provinces and mountainous regions face chronic shortages, particularly in specialties like anesthesiology, intensive care, and oncology. The government has used a mix of financial incentives, mandatory rural service periods for new graduates, and telemedicine outreach from central hospitals to provincial and district facilities to try to narrow this gap, with mixed success so far. Rising private-sector salaries are also creating a pull factor drawing experienced clinicians away from public hospitals in major cities, which is prompting public facilities to look at compensation reform and career-development incentives to retain staff even as the private sector expands.[22,23,24]

## Education

Vietnam trains its physicians through a network of medical universities led by Hanoi Medical University and Ho Chi Minh City University of Medicine and Pharmacy, both long established institutions that anchor postgraduate specialty training as well as undergraduate medical education, alongside a growing number of provincial and private medical schools established over the past two decades to expand training capacity. Medical training in Vietnam has traditionally followed a six-year undergraduate model rather than the separate undergraduate-then-medical-school pathway used in the United States, with postgraduate residency and specialization pursued afterward through hospital-based training programs. The government and leading academic hospitals have been investing in partnerships with foreign medical schools and hospital systems to modernize

curricula, expand simulation-based training, and build subspecialty expertise in areas like interventional cardiology, oncology, and organ transplantation, reflecting an explicit strategy of reducing the number of complex cases that historically traveled abroad, often to Singapore or Thailand, for treatment. Continuing medical education and specialist certification are increasingly emphasized as the government tries to raise the ceiling of what domestic hospitals, particularly top-tier public and private facilities, can safely and reliably treat in-country.[25,26]

## Healthcare Outlook

Vietnam's push toward free public healthcare is not a single program with one target date; it is a set of overlapping initiatives the government has rolled out in phases, each with a different scope and timeline, and understanding the difference is essential to reading official statements accurately. Communist Party General Secretary To Lam has framed healthcare as a national priority and a human right, and the roadmap that follows starts with free annual health check-ups: from 2026, the government intends to extend preventive screening coverage nationwide, aiming for 90 percent of the population to receive annual check-ups by 2030. That is a prevention and screening benefit delivered mainly through the existing commune and district health station network, not a promise to cover hospital treatment costs. A second, larger phase targets free hospital care itself: officials have said that between 2030 and 2035 the country aims to eliminate hospital fees at the point of care for the entire population, a goal that depends on a planned overhaul of the Law on Health Insurance and reformed provider payment mechanisms that the Ministry of Health intends to complete within that same five-year window. Fee waivers for the most vulnerable groups, the poor, the elderly, children, people with disabilities, and patients with catastrophic illness, are meant to arrive earlier, inside the 2026 to 2030 window, ahead of the universal rollout. Layered on top of both nearer-term phases is a longer-horizon national vision, expressed in Party and government messaging, of reaching health outcomes and universal free care comparable to those of developed countries by 2045; that date functions less as an operational deadline than as the end state the 2030 and 2035 milestones are meant to build toward. Put simply: free preventive checkups by 2030, free hospital care for everyone by 2035, and parity with developed-country health systems as the 2045 aspiration, with earlier fee relief for vulnerable groups bridging the gap. For an institution like Cedars-Sinai International, this timeline matters less as a threat to private and international care, since public free-care expansion targets basic and routine treatment, and more as a signal of sustained government commitment to health system modernization, which tends to raise standards, expand insurance-funded demand, and create openings for partnership on specialty and complex care that the domestic system is still building capacity to handle.[27,28,29]

## Competitive Landscape

Vietnam's healthcare market sits at an inflection point where domestic private hospital groups, foreign invested facilities, and regional hospital operators are all competing for the same pool of higher-income and insured patients, while outbound medical tourism to Singapore, Thailand, and increasingly South Korea continues to drain complex and high-value cases that Vietnamese hospitals are still building the capacity to treat. Domestic conglomerate-backed chains like Vinmec have invested heavily in building internationally accredited flagship hospitals in major cities, explicitly positioning themselves to capture patients who would otherwise travel abroad, while smaller private operators compete more on convenience, shorter wait times, and English-language services for the expatriate community. The clearest documented US academic-medical-center

engagement in the market is between Cleveland Clinic and Vinmec: a second Vinmec hospital joined the Cleveland Clinic Connected network in May 2024, and in April 2025 Cleveland Clinic and Vingroup announced collaboration on the clinical model for the new Vinmec Can Gio International General Hospital. Mayo Clinic has had smaller-scale educational involvement, with physicians supporting a gynecologic oncology fellowship in Da Nang since 2017. No confirmed partnerships were found for Johns Hopkins, MD Anderson, Memorial Sloan Kettering, Houston Methodist, UCLA, NewYork-Presbyterian, University of Pittsburgh Medical Center, Mass General Brigham, or Northwestern Medicine with Vietnamese institutions as of this research, so these should be treated as not established unless Cedars-Sinai has proprietary knowledge otherwise. The Thomson Medical acquisition of FV Hospital's parent company, finalized in January 2024, exemplifies a broader trend of regional players buying into the market directly rather than only receiving outbound patients, and further consolidation among mid-sized private hospital groups is widely expected as investors look to build scale and negotiating leverage with insurers and suppliers. For Cedars-Sinai International, this creates a window to engage Vietnamese partners, whether public academic medical centers seeking training and research collaboration or private hospital groups seeking accreditation support, clinical protocols, and specialty program development, while government reform is actively raising the ceiling for what the domestic system aims to deliver.[30,31,32,33]

# Recent Developments

**2026-04** The 16th National Assembly convened with 500 members following 2026 elections, an increase from the 498-member 15th National Assembly, reflecting adjusted electoral district allocations.[5]

**2026 (rollout begins)** Rollout begins of nationwide free annual preventive health check-ups, the first phase of the government's multi-year free healthcare roadmap, targeting 90 percent population coverage by 2030.[34,35]

**2025** Vietnam carried out a large-scale consolidation of provincial and district administrative units, reducing the number of administrative layers nationwide, with direct implications for how provincial health departments and hospital networks are organized and funded.[36]

**2025-04-18** Cleveland Clinic and Vingroup announced collaboration on the clinical model of care for the new Vinmec Can Gio International General Hospital, extending Cleveland Clinic's existing relationship with Vinmec's flagship facilities.[37]

**2024-01** Thomson Medical Group completed its approximately 381.4 million dollar acquisition of FV Hospital's parent company, closing a deal first agreed in 2023, in one of the largest private healthcare transactions in Vietnam's history.[38]

# CSI Assessment

*Everything below comes from Cedars-Sinai's internal data (patient referral records, finance, relationship history) and human judgment, not from the research agents. Fields left blank are marked and must be completed by the analyst before this profile is used.*

## Patient Volume & Revenue (most recent FY)

**[MANUAL INPUT REQUIRED]**

## Referral Sources

**[MANUAL INPUT REQUIRED]**

## Relationship History

**[MANUAL INPUT REQUIRED]**

## Complications

**[MANUAL INPUT REQUIRED]**

## Key Contacts

**[MANUAL INPUT REQUIRED]**

## Opportunities

**[MANUAL INPUT REQUIRED]**

## CSI Recommendation

**[MANUAL INPUT REQUIRED]**